

Peripheral venous cannulation in a confused patient

Sir,

I would like to report a modification of the technique for inserting a peripheral venous cannula (January 1990, p.67), which has saved this junior doctor a number of disturbed nights when trying to maintain an infusion in a confused patient.

The cannula is inserted as described in the article and firmly fixed to the skin, and a bung containing a central diaphragm is screwed onto an 18 G needle which is then pushed through the bung's diaphragm and thus into the cannula, in the same way as for intermittent injections. The giving set tube is then taped lightly to the patient's arm and the infusion commenced.

If the patient pulls on the irritating tube, the needle is pulled out of the cannula but

the cannula remains in the vein, and it is then a simple matter to recommence the infusion (using a fresh needle) without further trauma to the patient or doctor.

The risk of the patient damaging himself with the bare needle is minimized by taping the tubing down lightly so that the avulsed needle falls away from the patient.

Clearly this is no substitute for the diagnosis and treatment of the cause of the confusion or for close observation of the patient, but in practice this has proved to be a useful technique which deserves wider publication.

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An age of little progress in anxiety control

Sir,

The 20th century has been an age of attempts at anxiety control. Remarkable progress in our knowledge of anxiety in psychoanalytic, psychological, behavioural, cognitive and biological terms has been made. The number of treatment modalities available to us is amazing. More than 500 types of psychotherapies and 50 different drugs and innumerable research publications theoretically mean that no psychiatrist will ever be more confident than he is now.

However, Fisher's article on the causes and control of anxiety (September 1990, p.194) generated anxiety as I read it through. There was little to argue with in the concluding remarks of the author: 'Drug control may be an effective measure in the short term because it has the potential to break the positive feedback loop. In the end, however, it may create rebound effects or secondary stresses.' The most anxiety-generating thought is that despite the numerous therapies available we are still no closer to controlling anxiety, however confident we may be.

In general, there are two approaches to

the control of anxiety. The first is to prevent the environmental cues triggering the anxiety. But such an approach imposes innumerable restrictions on the individual with an inevitable lowering of efficiency. The second approach is to try to control the manifestations of anxiety, with the help of the various psychological, cognitive and organic therapies available to us. The problem with such therapies is that the anxiety-generating stimulus is still prevalent in the environment. The therapist has no choice but to hope and speculate that following the course of therapy the anxiety-generating stimulus will no longer trigger the anxiety.

Unfortunately these are very unscientific approaches to a scientific problem. As we enter the last decade of this century, let us hope that we will conquer anxiety by the end of it, so that whereas this century has been an age of anxiety, the next will be the age of anxiety control.

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Premenstrual syndrome: an alternative view

Sir,

There is an alternative view of premenstrual syndrome (PMS) to that expressed by Watson and Studd (October 1990, p.286).

PMS is not limited to ovular cycles, but also occurs during anovular cycles, especially at puberty, menopause and after bilateral oophorectomy (Dalton, 1984). The unique characteristics of progesterone receptors have been overlooked, for it is the failure of progesterone receptors to bind to progesterone in the blood which lies at the root of PMS (Dalton, 1990). After an initial dose of progesterone the progesterone receptors require a 40 times higher dose of progesterone before they form a hormone-receptor complex (Dalton, 1984). This explains the high level of progesterone required in the treatment, and also failure of the many low dose clinical trials. Progesterone receptors do not act in the presence of noradrenaline (Nock, 1986) and therefore a diet to prevent a drop in blood glucose is essential. The man-made progestogens, alien to the human body, lower blood progesterone level (Dalton, 1984) and do not bind to progesterone receptors (Dalton, 1984), therefore progestogen administration causes PMS.

The 10% incidence of hysterectomy that follows the oestrogen and progestogen therapy recommended by Watson and Studd, is wholly unacceptable. Progesterone is the drug of choice in the treatment of PMS and high doses are required (Dalton, 1984; Nock, 1986).

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Dalton K (1984) *The Premenstrual Syndrome and Progesterone Therapy*. Heinemann Medical Books, London

Dalton K (1990) The aetiology of premenstrual syndrome is with the progesterone receptors. *Medical Hypothesis* 31: 323-7

Nock B (1986) Noradrenergic regulation of progestin receptors: new findings, new questions. Reproduction: a behavioral and neuroendocrine perspective. *Ann NY Acad Sci* 474: 415-22

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